

CRM NO 190643000231	Name B/o. THENDRAL 586829	Father Name KISHORE	Lab ID 21200108317
Package BS 6	Gender MALE	DOB/Age 29/10/2022 (1 Mth 14 Days 3 Hrs)	Referred By Dr. RAMAKRISHNAN
Hospital name SKS Hospital	City SALEM	Pre Term / Full Term Full term	Baby Weight (Kg) 3.000
Specimen Source Heel Prick	Specimen Notes Acceptable	Collection Date & Time 13/12/2022 03:00	Received Date & Time 14/12/2022 15:00
Date of Report & time 16/12/2022 14:02	Blood Transfusion NO	Transfusion Date NO	Special Feeds / IVF / TPN/ Supplements NO

INBORN ERRORS OF METABOLISM (IEM) SUMMARY REPORT

Sr. No.	Test Methodology	Result	Test Type	Page
1	FLUOROMETRIC ENZYME IMMUNOASSAY SCREENING REPORT	Negative	Screening	2

DISCLAIMER

All investigations have their limitations which are imposed by the limits of sensitivity and specificity of individual assay procedures as well as the quality of specimen received by the laboratory.

It is presumed that the received specimen belongs to the patient named or identified in the test request form.

Partial reproduction of this report is not permitted.

It is important to monitor baby's health and take a proactive approach should there be any unhealthy condition(s) within or beyond scope of this test.

A negative screening assay result does not rule out the possibility of an underlying metabolic / genetic disease.

The metabolites and health conditions mapping is not mutually unique and there can be other forms in which an abnormality may be apparent. Similarly, the health condition may not exclusively be an outcome of the disorders in consideration.

If the newborn is premature, retest is suggested when adjusted gestational age is 40 weeks.

HB-Variant hemoglobins may require further testing by the laboratory to diagnose.

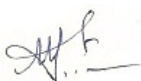
HB-Presence of Hb Bart's with any newborn screen result indicates the presence of alpha thalassemia of unknown severity.

HB-If an increased amount of adult hemoglobin is noted, the infant is presumed to have been transfused (Including Intrauterine transfusion).

HB-Carriers of beta thalassaemia and HbLepore cannot be excluded at this age.

Investigations performed is not intended to replace the medical advice and or treatment obtained from a qualified healthcare practitioner as it doesnot cover all the metabolic disorders

HB (Covered under Hemoglobinopathies package)



Dr. Mahesh Hampe , MD

Lab Director

REPORT- SUMMARY

Observations:

All the analytes are within the normal range.

Interpretation:

The tested sample is screened **NEGATIVE** for the list of disorders tabulated in page 2.

Recommendation:

Please correlate the results with other clinical and diagnostic findings.

Initial Report

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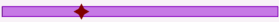
Duplicate Report

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Revision Report

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Version No

Disorder	Parameter	Result*	Result Value	Reference	Remarks
					
Glucose 6 Phosphate Dehydrogenase deficiency	G6PD		22.7	NLT 3 U/dL	Normal
Classical Galactosemia	T GAL		2.5	NMT 25 mg/dL	Normal
Congenital Hypothyroidism	TSH**		3.1795	NMT 10 µU/mL	Normal
Congenital Adrenal Hyperplasia	17-OHP		11.087	NMT 55 ng/mL	Normal
Phenylketonuria	PHE		71.04	NMT 168 µM/L	Normal
Profound Biotinidase deficiency	BTD		150	NLT 40 U	Normal

* This Graph represents the value corresponding to results values. Each box represents the reference value. Results value is shown as dot.

* **NMT** - Not More Than, **NLT** - Not Less Than. ** TSH values are represented in Serum unit (Conversion factor: 1 µU/mL blood = 2.22 µU/mL serum).

G6PD - Glucose-6-Phosphate Dehydrogenase, T GAL - Total Galactose (galactose and galactose-1-phosphate), TSH** - Thyroid Stimulating Hormone, 17-OHP - 17a-OH-progesterone, PHE - Phenylalanine, BTD - Profound Biotinidase

REFERENCES

- <https://www.aap.org/en-us/advocacy-and-policy/aap-health-initiatives/PEHDIC/Pages/Newborn-Screening.aspx>
- Sudhanshu S, Riaz I, Sharma R, Desai MP, Parikh R, Bhatia V. Newborn Screening Guidelines for Congenital Hypothyroidism in India: Recommendations of the Indian Society for Pediatric and Adolescent Endocrinology (ISPAE) – Part II: Imaging, Treatment and Follow-up. The Indian Journal of Pediatrics. 2018 Jun 1;85(6):448–53.

** End Of Report **